Committee: World Health Organization (WHO)

Topic: The Question of Reducing Inequalities in Global Healthcare Systems in Developing Countries Through Enhanced Global Partnership

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Introduction

Accessing the quality healthcare is widely recognized as a basic human right, but large differences still exist between healthcare systems around the world. While people in many developed countries can generally access modern hospitals, medical technology, and trained healthcare workers, many developing countries lack these resources. Hospitals may not have enough equipment, medicines, or beds, and people living in rural areas often need to travel long distances to receive treatment. As a result, illnesses that could be treated easily may become more serious because patients cannot receive medical care at the right time.

Another major challenge is the shortage of healthcare workers. Doctors, nurses, and other medical professionals are essential for providing quality healthcare, but many developing countries do not have enough trained workers. The problem can become worse because of “brain drain,” in which skilled healthcare workers move to wealthier countries for higher salaries, better working conditions, and greater career opportunities. Although this decision is understandable for individuals, their home countries may lose workers who are urgently needed. Therefore, improving salaries, working environments, medical education, and career opportunities is important for encouraging healthcare workers to remain and work where they are needed.

Healthcare costs also create serious inequality. Universal Health Coverage (UHC) aims to ensure that everyone can receive necessary healthcare without facing serious financial difficulties, but this goal is difficult to achieve in countries with limited resources. Low-income families may avoid visiting hospitals or buying medicine because they cannot afford the cost. In serious cases, one illness can force a family to borrow money or use all of its savings. This can create a cycle in which poverty leads to poor health, while poor health makes poverty even worse. For this reason, governments need healthcare systems that make essential treatment more affordable and provide stronger financial

protection for their citizens.

Reducing inequality also requires improving basic healthcare services and infrastructure. Strong primary healthcare can provide vaccinations, regular checkups, health education, and treatment for common illnesses before they become more serious. This can improve people's health while reducing pressure on large hospitals. Technology can also help close the healthcare gap. For example, sharing medical knowledge and technology can help developing countries improve their healthcare services, while telemedicine may allow people in remote areas to receive medical advice without traveling long distances. However, these solutions require long-term investment in hospitals, internet access, medical equipment, and other basic infrastructure.

These problems are not limited to individual countries because weak healthcare systems can also threaten global health. Infectious diseases can spread across national borders quickly, especially when countries are unable to detect and control outbreaks effectively. A lack of testing equipment, trained workers, and medical supplies can make it difficult to respond to a health emergency. Therefore, international cooperation is necessary. Developed countries and international organizations can support developing countries through financial aid, medical training, research cooperation, and technology transfer. However, this support should focus on building sustainable healthcare systems rather than providing only short-term assistance, and each country's own needs and circumstances should be respected.

In conclusion, healthcare inequality remains a major global problem caused by differences in resources, healthcare workers, infrastructure, and financial protection. No single solution can solve all of these problems. Stronger primary healthcare, better support for medical workers, affordable treatment, improved technology, and long-term international partnerships are all necessary. Organizations such as the WHO and cooperation between governments can help countries share resources and knowledge to make healthcare systems stronger. Although achieving UHC will take time, continued global partnership can help ensure that access to quality healthcare depends less on a person's income or country of birth and becomes a fairer opportunity for people around the world.

Definition of Key Terms

Social determinants of health

Social determinants of health refer to non-medical conditions where people are born, grow, live, work and age. It also includes education, economic status, physical environment, and social support, which influence their overall health outcomes before treatments are made from clinics. The World Health Organization notes that these factors act as root causes of health inequalities, and as a person's physical surroundings and economic status heavily shape their chances of getting sick or staying healthy long before ever seeing a doctor.

Health Equality

Health equality means giving everyone the same healthcare resources, services, and opportunities, regardless of their income, background, or health conditions. It focuses on treating everyone equally by providing the same level of support. However, giving everyone the same resources does not always lead to fair results because some people face more difficulties in accessing healthcare than others. This is why it is important to understand the difference between health equality and health equity.

Health equity

Health equity means making sure that everyone has a fair chance to achieve good health. Some people face more difficulties in getting medical care because of their income, where they live, or their living conditions. For this reason, they may need more support or resources than others. Unlike health equality, health equity focuses more on providing support based on people's needs so that unfair differences in health can be reduced.

Brain Drain

Brain Drain refers to the movement of skilled workers, such as doctors, nurses and medical researchers, from their home countries to other countries in search of better salaries, working conditions, and career opportunities. This often happens when wealthier countries can offer better payments and access to more advanced medical technologies. As a result, developing countries may

lose healthcare workers who are needed to provide medical services to their communities. This can make existing staff shortages worsen and make it harder for people to receive proper medical care.

Universal Health Coverage (UHC)

Universal Health Coverage or the UHC, is a goal supported by the World Health Organization and included in the United Nations Sustainable Development Goals. It aims to make sure that everyone can receive the healthcare services they need without facing serious financial difficulties. These services include disease prevention, treatment, rehabilitation and other forms of medical care. In order to achieve UHC, countries need healthcare systems that make essential services affordable and help protect people from financial problems caused by medical expenses.

Primary Health Care (PHC)

Primary Health Care or PHC, focuses on providing basic medical services in local communities. These services include vaccinations, regular checkups, health education, and treatment for common illnesses. By helping people receive medical care early, PHC can prevent health problems from becoming more serious and reduce the need for hospital treatment. It is especially important in rural areas, where hospitals may be far away and access to medical services is limited. Improving primary healthcare can therefore help reduce differences in access to healthcare.

Background Information

Historical Background

Healthcare inequality has become an important international issue over time. The idea that health is a basic human right was included in the WHO Constitution, which was adopted in 1946. In the 1970s, international efforts to improve health for all became more important. In 1977, the World Health Assembly adopted the goal of "Health for All." Then, in 1978, WHO and UNICEF jointly organized the International Conference on Primary Health Care, where the Declaration of Alma-Ata was adopted. The declaration pointed out the large differences in health between developed and developing countries, as well as within countries. It also stated that these differences were a concern for all countries and emphasized primary health care and international cooperation as important ways to improve global health.

Since then, the international community has continued to make efforts to reduce healthcare inequality. In 2005, WHO established the Commission on Social Determinants of Health to study how social and economic conditions can affect people's health. Universal Health Coverage (UHC) later became an important part of the Sustainable Development Goals. In 2018, the Declaration of Astana renewed the international commitment to primary health care and emphasized its importance in achieving UHC.

Impact of COVID-19 on Healthcare Inequality

The COVID-19 pandemic made healthcare inequality even worse in many parts of the world. During the pandemic, hospitals and healthcare workers had to focus heavily on treating COVID-19 patients. Because of this, other important medical services, such as regular vaccinations and treatment for diseases like cancer, HIV, tuberculosis, and malaria, were sometimes delayed or interrupted. Countries with weaker healthcare systems often had more difficulty dealing with these problems. COVID-19 also showed the difference between countries with strong and weak healthcare systems. Some countries had enough money and resources to get vaccines, medical equipment, and healthcare services more quickly. However, many poorer countries had fewer resources and faced more difficulties. The pandemic also caused economic problems, making it harder for some people and families to pay for medical care.

Overall, COVID-19 showed that healthcare inequality can become even worse during a global health crisis. People and countries that already had limited access to healthcare were often affected more seriously. This showed the importance of building stronger healthcare systems and improving international cooperation so that countries can respond better to future health emergencies.

The Structural Gap in Global Healthcare Infrastructure

The imbalance in global healthcare infrastructure remains one of the major issues which interfere with humanitarian and economic developments. A high number of low to middle income countries tend to suffer from systemic underfunding, including severe shortages of qualified medical personnel and are scarce in basic infrastructure. Rural and overpopulated areas are facing these inequities, often being way further away even from facilities which lack essential materials such as surgical instruments, electricity, clean water, secure cold-chain distributions and medication. These types of isolations are one of the many reasons for delay in critical treatments, a systemic crisis reflected in global health data. According to joint monitoring from the World Health Organization and the World Bank, 4.6 billion people lack access to essential healthcare costs, leading 300 million people to extreme poverty. Also, the data shows that an average of $3,038 per person is spent in high-income nations, whereas low-income countries spend approximately $45. Considering these differences of financial difficulties, the World Health Organization projects a global shortfall of 11 million health workers by 2030, heavily concentrated in Sub-Saharan Africa and Southeast Asia. Without basic infrastructure and lack of medical supplies, the difference of healthcare service between more and less developed countries will rise.

Brain Drain and Healthcare Worker Shortages

Another important cause of healthcare inequality is the shortage of healthcare workers in many developing countries. Doctors, nurses, and other medical workers are necessary for providing basic healthcare, but some countries do not have enough trained workers. One reason for this is the “brain drain.” This happens when skilled healthcare workers move from developing countries to wealthier countries in search of better salaries, working conditions, and career opportunities. In some cases, countries spend time and money training healthcare workers, but later lose them when they move abroad.

Although these workers may have personal reasons for moving, their departure can make healthcare systems in their home countries weaker. Hospitals and clinics may have fewer doctors and nurses, which can make it harder for people to receive medical care. This problem is especially serious in rural areas, where there may already be a shortage of healthcare workers. As a result, patients may have to travel long distances or wait longer to receive treatment. During disease outbreaks or other health emergencies, a shortage of healthcare workers can make it even more difficult for a country to respond effectively.

To reduce this problem, countries need to improve working conditions and provide better support for healthcare workers. They can offer fair salaries, safer working environments, and more opportunities for education and career development. International cooperation can also help by supporting medical education and training in countries with healthcare worker shortages. Wealthier countries can also consider the effects of recruiting healthcare workers from countries that already have serious shortages. By strengthening and keeping the local healthcare workforce, developing countries can improve access to healthcare and reduce healthcare inequality.

The Role of Digital and Smart Health Technologies

Smart health infrastructure such as mobile health applications, AI-driven diagnostic platforms, and Internet-of-Things (IoT) monitored supply chains offer alternative approaches towards spatial barriers in medical fields. An example for this is the cold-chain temperature control failures, which cause substantial vaccine wastage globally. IoT-enable thermal sensors, on the other hand, prevent these losses by continuously monitoring the conditions of vaccines and further adjusting the temperature. Similarly, telemedicine networks and automated diagnostic algorithms can be extended to specialist care in rural areas without the need of mass constructions.

However, the mass use of digital devices still remains limited toward one third of the population, which doesn’t have direct or none connections to the internet. According to the International Telecommunication Union (ITU), 2.6 billion people (31.3 percent of the global population) remain completely offline, and 96 percent of these people are residing in developing countries. The investments in foundational energy and telecom grids, smart health deployments has risks upon bringing actual help towards those who are disconnected to the digital infrastructure such as the elderly, rural communities, and low-income families.



UN *Sustainable development goal 3 (United Nation, 2015)*

Possible solutions

Funding Between Nations

One of the main obstacles in solving the issue above is the fact that less developed countries lack financial resources needed to improve healthcare systems and provide enough medical supplies. Building hospitals, training healthcare workers, and providing essential medicines all require a large amount of money. Therefore, developed countries can support these countries through different types of international funding. This support can be especially important for countries facing both economic difficulties and weak healthcare systems.

First, developed countries can provide direct financial aid or grants to help build hospitals, improve primary healthcare services, and provide essential medicines. Grants can be especially helpful because developing countries do not have to repay the money. This allows governments to focus more of their resources on improving healthcare. Second, international organizations and developed countries can provide low-interest loans for long-term healthcare projects, such as building medical facilities or improving healthcare infrastructure. Another possible solution is debt relief, which can reduce the financial burden on developing countries and allow them to invest more money in healthcare. Developed countries could also use a matching fund system, where they provide additional funding based on how much a developing country invests in its own healthcare system. This could encourage countries to continue investing in their own healthcare services. Finally, countries can contribute to international health funds that provide support during disease outbreaks and other health emergencies, allowing countries with fewer resources to respond more quickly.

In addition, governments can work with private companies and international organizations to fund healthcare projects. Public-private partnerships allow governments and private companies to share resources, knowledge, and responsibilities. For example, private companies could help provide medical equipment or develop digital healthcare services, while the governments ensure that these services remain accessible to the public. To make sure that international funding is used properly, governments and organizations should publish reports on how the money is spent and evaluate whether the projects improve healthcare access. These measures can help ensure that financial support reaches the communities that need it most and contributes to long-term improvements.

Enhancing the Partnership and Technological Cooperation

Strengthening partnerships between developed and developing countries is another important way to reduce healthcare inequality. Many developing countries lack access to suitable medical equipment, modern healthcare technology, and the technical knowledge needed to improve medical services. Through international cooperation, countries can share medical technology, research, and knowledge to help strengthen healthcare systems. However, technology transfer should involve more than simply sending equipment to another country. It should also include training local healthcare workers to use the technology safely and teaching technicians how to maintain and repair it.

Furthermore, medical technologies should be suitable for the needs and conditions of each country. For example, portable medical devices may be more useful in rural areas where hospitals have limited equipment and resources. Partner countries can also support the development of local skills so that healthcare systems can continue operating after the initial assistance ends. Public-private partnerships can contribute to this process by allowing governments, international organizations, and private companies to work together on developing and providing affordable medical technologies. Through long-term cooperation, developing countries can improve healthcare services and become less dependent on outside support.

Strengthening the Healthcare Workforce

Another possible solution in reducing healthcare inequality is by strengthening the healthcare workforce in developing countries. Many countries do not have enough trained doctors, nurses and other healthcare professionals, especially in rural and remote areas. Even if hospitals and medical equipment fulfill the standard, it is difficult to provide proper healthcare without enough qualified workers. The government can invest more in medical education and training programs to encourage healthcare workers in underserved areas by providing better support and working conditions.

In addition to this, governments can develop policies to help healthcare workers to remain in rural and remote areas for the longest time possible. For example, they can provide appropriate financial or non-financial incentives, training opportunities, and professional support. It's important to strengthen the healthcare workforce as systems need both facilities and qualified people for medical services. By increasing the number of trained healthcare workers and improving their distribution, countries can help many more people to get essential healthcare services.

Developing Facilities of Health Technologies

Another possible solution to reduce global healthcare inequality is improving healthcare technology and facilities in developing countries. Many healthcare systems in these countries still lack modern medical equipment, diagnostic tools, and digital healthcare systems, especially in rural areas. Even when trained medical professionals are available, it can be difficult to provide proper treatment without the necessary equipment and facilities. Therefore, international organizations and governments can work together to improve healthcare facilities, share medical technologies, and support the local production of basic medical equipment. This can also help developing countries reduce their dependence on expensive imported equipment.

However, providing new equipment alone is not enough. Countries also need trained workers who can operate, maintain, and repair medical devices. International partnerships can support training programs for local technicians and engineers to ensure that equipment continues to function properly. Digital health services can also help people access medical advice when hospitals are far away. For example, telemedicine allows patients to consult healthcare professionals remotely, while digital health platforms can help healthcare workers share medical information. In areas with limited electricity or internet access, governments should also work to improve basic infrastructure so that these technologies can be used effectively. Combining suitable equipment, local production, technical training, and digital health support can help countries build stronger healthcare systems over time.

Major parties involved

United States of America

The United States of America is a critical party in global health politics as the primary funder of major multilateral health initiatives, including PEPFAR and Global Health Security Agenda (GHSA) program. Through the U.S. Agency for International Development (USAID) and the Centers for Disease Control and Prevention (CDC), it drives global epidemic preparedness, vaccine distribution, and biomedical research. They also are in the process of negotiating bilateral funding frameworks, and leveraging public-private medical innovation, playing a key role overall in solving such crises.

French Republic

The French Republic plays a central role in shaping multilateral health governance, particularly through its leadership in Universal Health Coverage (UHC) initiatives and contributions to the Global Fund to Fight AIDS, Tuberculosis and Malaria. France's national healthcare system, regulated under the Code de la santé publique, emphasizes equitable care delivery and international development assistance via Agence Francaise de Developpement (AFD). Delegates can engage France on funding public health infrastructure in developing nations and expanding regional health worker training networks.

People's Republic of China

The People's Republic of China exerts significant influence on global public health through its " Health Silk Road" framework and bilateral medical cooperation across the Global South. Governed nationally by the National Health Commission (NHC), China specializes in large scale healthcare infrastructure deployment, emergency medical team deployments, and active pharmaceutical ingredient (API) supply chain management. For delegates, China is a critical actor for establishing affordable medical equipment supply pipelines and developing localized hospital infrastructure.

Federal Republic of Germany

The Federal Republic of Germany is a leader in global pandemic prevention and sustainable health funding, working through its Federal Ministry of Health (Bundesministerium für Gesundheit) and its development agency, GIZ. Through its Global Health Strategy, Germany stresses the importance of working with international groups like the World Health Organization (WHO), improving digital health tools, and training healthcare workers. Germany suggests a solid framework for improving health security and building healthcare capacity in lower-income countries.

Federal Republic of Nigeria

The Federal Republic of Nigeria is an important voice for developing nations in Sub-Saharan Africa. Guided by the Federal Ministry of Health and the Nigeria Centre for Disease Control (NCDC), Nigeria works to improve healthcare access through policies like the National Health Act and Basic Healthcare Provision Fund (BHCPF). Nigeria represents the real challenges which many of the developing countries face, such as healthcare worker shortages, a lack of domestic funding, and the urgent need for fair access to medical supplies and technology.

Republic of South Africa

The Republic of South Africa is a key regional leader working to fix healthcare inequalities at home through its National Health Insurance (NHI) plan. Led by its National Department of Health, South Africa actively pushes for changes to global trade rules so that developing countries can produce their own vaccines and medicine locally. South Africa is an important partner for the developing nations in standing up for local drug production, cheaper medical supplies, and stronger cooperation among countries.

Timeline Of Events

Date	Description of Event
Alma-Ata Declaration, 1978	Countries agree that primary healthcare is essential for achieving better health and reducing inequalities.
Paris Declaration on Aid Effectiveness, 2005	Developed and developing countries agreed to make international aid more effective through stronger cooperation, coordination, and mutual accountability. This helped create a framework for supporting developing countries more effectively.
WHO supports Universal Health Coverage, 2012	The UN and WHO increase efforts to promote Universal Health Coverage (UHC), aiming to provide healthcare without financial hardship.
COVID-19 Pandemic, 2020	The pandemic reveals major inequalities in access to healthcare, vaccines, and medical supplies between developed and developing countries.
COVAX expands global vaccine cooperation, 2020	COVAX is launched to support more equal access to COVID-19 vaccines, especially for lower-income countries.
WHO Pandemic Agreement, 2025	WHO member states adopt a global agreement to strengthen international cooperation and improve a more equal response to future pandemics.

UN Involvement, Resolutions, Treaties and Events

Transforming Our World: The 2030 Agenda for Sustainable Development, 25 September 2015

- The United Nations adopted the Sustainable Development Goals (SDGs) to address global challenges through international cooperation.
- SDG 3 aims to ensure healthy lives and promote well-being for people of all ages.
- Target 3.8 specifically calls for achieving Universal Health Coverage (UHC), including access to quality healthcare and financial protection.
- This resolution provides an important global framework for reducing healthcare inequalities, especially in developing countries.

Political Declaration of the High-Level Meeting on Universal Health Coverage, 10 October 2019

- UN Member States reaffirmed their commitment to achieve Universal Health Coverage by 2030.
- The resolution recognizes that many people still face financial barriers and unequal access to essential healthcare services.
- It calls for stronger healthcare systems and international cooperation to ensure that no one is left behind.
- It is directly related to reducing inequalities in healthcare systems between developed and developing countries.

Strengthening WHO Preparedness for and Response to Health Emergencies, 31 May 2021

- The COVID-19 pandemic showed major inequalities in access to healthcare, medical supplies, and vaccines.

- WHO Member States adopted this resolution to strengthen global cooperation and health emergency preparedness.

- It encourages countries to improve healthcare systems, research, surveillance, and information sharing.

- Stronger international partnerships can help developing countries respond more effectively to future health emergencies.

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